AM-06 Suspected Stroke - Tenecteplase (TNK) Transfer

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Interfacility transfer after hospital-administered TNK for suspected acute ischemic stroke. TNK is a single IV bolus; there is no thrombolytic infusion to continue, titrate, or stop.

Post-TNK transfer and complication pathway

1. Verify transfer	Accepted sending/receiving plan, exact TNK bolus time, patient weight/dose, last known well, baseline neuro examination, and written orders.
2. Before departure	Airway, glucose, IV sites, BP below 180/105, continuous monitor, NPO, receiving acceptance, and destination verified.
3. Every 15 min	BP, GCS, pupils, focal findings, airway, SpO2, and bleeding/angioedema symptoms. Increase frequency if unstable or on an authorized infusion.
4. Continue ordered infusion	Only a sending-facility antihypertensive infusion with written order and accepted pump parameters. Do not initiate or titrate a new antihypertensive under AM-06.
5. Deterioration	Severe headache, vomiting, acute hypertension, worsening deficit, seizure, bleeding, or angioedema: support ABCs, recheck BP/glucose, stop nonessential infusions, notify stroke facility and Medical Control.
6. Direct transport	No unnecessary invasive procedures or IM medication. Apply prolonged pressure after any essential vascular access.

Key treatment and disposition

Item	Approved action
BP target	Maintain BP below 180/105 for the first 24 hr after thrombolysis. Do not use the pre-thrombolysis 185/110 threshold as the post-TNK target.
TNK	Single hospital-administered IV bolus. Exact administration time must be known before departure.

Precise times, not estimates, are required for stroke handoff. Do not depart without the exact TNK bolus-administration time.

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SAFETY, LINKED PATHWAYS, AND RELEASE CONTROL

Safety checks

- Sudden neurologic deterioration, severe headache, vomiting, seizure, acute hypertension, or bleeding may indicate intracranial hemorrhage or edema.

- Angioedema can threaten the airway even without other allergic findings.

- Keep the patient NPO and avoid antiplatelets, anticoagulants, additional thrombolytic therapy, or unapproved IM medication unless specifically ordered and confirmed under policy.

- AM-04 is only a defined undifferentiated-CVA branch; post-TNK BP care is governed by written facility orders and this protocol.

Linked pathways

Pathway	Use when
UP-04	Ongoing neurologic/reversible-cause assessment
AM-05	Hypotension or inadequate perfusion
AR-01	Airway protection/ventilatory support
UP-13	Seizure
AM-07	Peri-arrest deterioration

Documentation and release control

- Document focused findings, BP/rhythm/ECG when applicable, all interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.